AC-11 TEAM-Focused CPR

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for coordinated adult cardiac-arrest resuscitation. This is the required team-performance companion to the rhythm-specific arrest pathways.

Team roles and two-minute cycle

1. Recognize arrest	Start CPR, apply pads/AED, BVM with oxygen, and start the scene timer.
2. Assign functions	Leader, compressor 1/2, airway, monitor/defibrillator, IV/medications, and recorder/timer.
3. High-quality CPR	100-120/min, depth 2-2.4 in, full recoil, minimal pauses, compression fraction above 80%; switch compressors every 2 minutes or earlier if fatigued.
4. Ventilation	30:2 before an advanced airway; then continuous compressions with 1 breath every 6 seconds. Avoid hyperventilation.
5. Two-minute cycle	Precharge when possible, rhythm/pulse check 10 seconds or less, shock if indicated, and immediately resume CPR.
6. Communicate + record	Closed-loop orders; announce rhythm, shocks/energy, medications, airway, EtCO2, roles, and key times.

Key treatment and handoff

Item	Approved action
ROSC	Open AC-10 and AC-13.
Persistent arrest	Continue rhythm-specific care and evaluate AC-12 eligibility; do not use a single finding alone to terminate.

No team member should perform a task that delays compression quality, early defibrillation, or the defined rhythm pathway.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Pads, rhythm analysis, airway placement, access, and medication must be coordinated around compressions.

- When staffing is limited, prioritize compressor, AED/monitor, BVM/airway, and leader/timekeeper roles; add functions as help arrives.

- Document role assignments, changes, compression-quality interventions, rhythm, shocks, medications, and time-stamped reassessments.

Linked pathways

Pathway	Use when
AC-01	Asystole/PEA
AC-03	Adult cardiac arrest
AC-09	VF/pulseless VT
AC-12	Termination of resuscitation

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.